

PRODUCT LITERATURE

BROVENT TABLET 8MG

Each tablet contains

Bromhexine HCl 8 mg

Preservative

Sodium Benzoate 0.1%w/w

Product description

Yellow coloured odourless round tablet with R11 and a score embossed on one side and TPI in the other side.

Pharmacodynamics Properties

Bromhexine is an oral mucolytic agent with a low level of associated toxicity. Bromhexine acts on the mucus at the formative stages in the glands, within the mucus-secreting cells. Bromhexine disrupts the structure of acid mucopolysaccharide fibres in mucoid sputum and produces a less viscous mucus, which is easier to expectorate.

Pharmacokinetics Properties

Absorption

Oral, well absorbed
Rapidly absorbed from the gastrointestinal tract
Peak plasma concentrations occur after about 1 hour following oral administration

Metabolism

Bromhexine undergoes extensive first-pass metabolism in the liver:
Ambroxol is a metabolite of bromhexine

Distribution

It is widely distributed to body tissues
Bromhexine is highly bound to plasma proteins
Bromhexine crosses the blood-brain barrier and small amounts cross the placenta.

Excretion

Bromhexine is excreted primarily in the urine as metabolites. Only small amounts appear as unchanged drug.
About 85 to 90% of a dose is excreted in the urine mainly as metabolites. Approximately 70% of an oral dose of bromhexine has been recovered in the urine within 24 hours
Other excretion: faeces, 4%

Elimination Half-life

It has a terminal elimination half-life of 13 to 40 hours

Indication

Reduction of sputum viscosity

Recommended dose

To be given 3 times a day
Adult : 1 - 2 tablets
Children (6 - 12 years): 1/2 - 1 tablets

Route of administration

Oral

Contraindications

Hypersensitivity to bromhexine or other components of the formulation.

Warnings and precautions

Very rare cases of chronically associated severe skin impairments such as Stevens Johnson Syndrome, Toxic Epidermal Necrolysis (TEN), Erythema Multiforme (EM) and Acute Generalized Exanthematous Pustulosis (AGEP) have been reported. In most cases, these could be explained by the severity of the underlying disease or concomitant administration of another drug. In the early stages of such severe skin reactions, initially only nonspecific flu-like symptoms appear, e.g. fever, arthralgia, runny nose, cough, and sore throat. If skin or mucous membrane damage occurs, seek medical advice immediately and discontinue treatment as a precaution.

Patients being treated with bromhexine should be notified of an expected increase in the flow of secretions.

The use of Bromhexine without medical supervision is not intended in patients who suffer from such condition chronically.

Medical advice should be sought if the symptoms last longer than 14 days and/or if the symptoms increase in spite of treatment with bromhexine.

Since mucolytics may disrupt the gastric mucosal barrier bromhexine should be used with care in patients with a history of peptic ulcer disease (gastric ulcer).

Care is also advisable in asthmatic patients.

Clearance of bromhexine or its metabolites may be reduced in patients with severe hepatic or renal impairment.

Bromhexine may increase the amount of antibiotic penetration. Antibiotics are medicines used to treat infections.

Interactions with other medicaments

Bromhexine may increase the concentration of concurrently administered antibiotics in bronchial secretions. No clinically relevant interactions with other medications have been reported

Pregnancy/Reproduction

Pregnancy

There is no data on the use of bromhexine in pregnant women. The effects if any, on the developing fetus are unknown. Until more information is available, bromhexine should only be used during pregnancy if the maternal condition justifies the potential risk to the fetus.

Lactation

Available evidence and/or expert consensus is inconclusive or is inadequate for determining infant risk when used during breastfeeding. Weigh the potential benefits of drug treatment against potential risks before prescribing this drug during breastfeeding.

Side effects

Dermatologic Effects: skin rash, urticaria,
Gastrointestinal Effects: nausea, epigastric pain, vomiting, and diarrhoea
Hepatic Effects: Transient elevations in serum aminotransferase levels
Neurologic Effects (Central nervous system): dizziness and headache
Renal Effects: Nocturnal enuresis
Others:
- anaphylaxis ,
- bronchospasm,; difficulty in breathing
- angioedema; swelling of the face, lips, mouth, tongue or throat which may cause difficulty swallowing or breathing.

ADR: Angioedema, pruritus, short breathes, overdose

Symptoms and Treatment of Overdose

No symptoms of overdosage have been reported in man to date. If they occur, supportive and symptomatic treatment should be provided.

Packing

Packing in blister, 10 tablets per blister.
Pack size: 10's, 30's, 100's and 1000's tablets.

Storage Condition

Store below 30°C, in a dry place, protected from direct light.
Keep away from reach of children.
Jauhkan daripada kanak- kanak.

Shelf life

3 years from date of manufacturer

Name and Address of Product Registration Holder/ Distributor:

ZONTRON PHARMACEUTICALS SDN.BHD. (445695-T)
Lot 10 & 11, PERDA Industrial Park,
Lorong IKS Simpang Ampat B,
14100 Simpang Ampat,
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Name and Address of Manufacturer:

TERAPUTICS SDN. BHD. (590500-W)
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